

SPMM
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Psychiatric Services

Paper B

Syllabic content 6

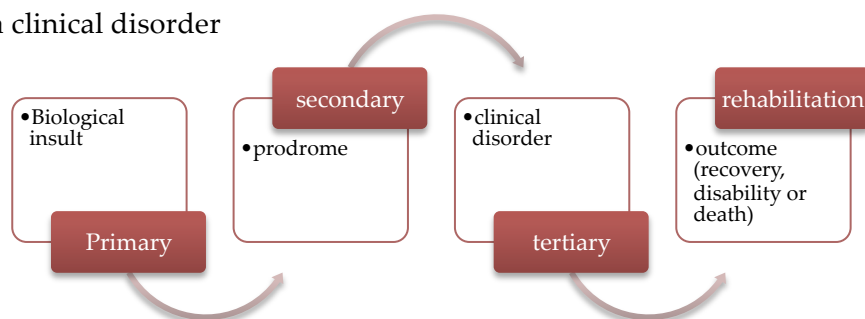
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1. Preventative strategies in psychiatry

Prevention psychiatry is the reduction of mental disorders and behavioral problems by (1) identifying risk and protective factors, and (2) Applying evidence-based interventions.

Most psychiatric disorders are thought to have a biological or sociological aetiology that produces the 'hit' for later development of the disorder. For a time after this insult, the patient may exhibit prodromal disturbances that are usually not picked up clinically. This prodrome later develops into full-blown clinically diagnosable disorder. This disorder can have various outcomes: disability, death or recovery. This natural course of a disease provides us with various nodes of intervention

1. Insult to prodrome node - averting a clinical disorder (primary)
2. Prodrome to diagnosis node – early diagnosis (secondary)
3. Diagnosis to outcome node - prevention of disability (tertiary)



Type	Aims	Methods	Examples
Primary Prevention	To reduce the incidence of the disease by preventing the development of new cases	Elimination of aetiological factors, increasing host resistance, the reduction of risk factors, and blocking modes of disease transmission	E.g. vaccines. Reducing adverse social factors for psychiatric disorders
Secondary Prevention	To reduce the total number of existing cases by more rapid effective intervention that shortens the duration of illness	Early identification and prompt treatment of illness	Screening programmes and early intervention, crisis support programmes e.g. mammography, pap smears
Tertiary Prevention	To reduce the prevalence of residual defects or disabilities that are consequences of the illness	It may not be possible to eliminate fully the sequel of the illness, but the goal of tertiary prevention is for individuals to reach their highest level of functioning.	Relapse prevention, rehabilitation

Prevention could result in

- Reduction of specific disorders: Reduced incidence and prevalence, delayed onset. e.g. Substance abuse, depression, PTSD

- Reduction of risky behaviours e.g. substance use, unsafe sex
- Reduction of negative outcomes: This will minimize adverse psychosocial impact of mental illnesses. e.g. suicide, teen pregnancy, school dropout, delinquency
- Promotion of mental health and wellness

In psychiatry currently as our knowledge of ‘insults’ is limited, most prevention is tertiary. Early intervention program in psychosis is an example of secondary prevention. Public health initiatives such as eradication of poverty, maintaining healthy diet etc. could prevent certain, at least milder forms of mental illnesses – these could be termed as **primary prevention strategies**. Interventions aimed at high-risk groups are usually **secondary preventions**. Rehabilitation can also be considered as a **tertiary prevention** aimed at reducing further disabilities. It is not appropriate to delay the initiation of rehabilitative techniques until acute treatment is complete, because it is not always clear whether the symptoms being treated are merely part of the acute process or will continue after acute treatment.

Institute of Medicine classification

There are two ways of classifying prevention strategies. In addition to the traditional public health definitions of primary, secondary, and tertiary prevention, a newer classification was put forth by Institute of Medicine in 1994. The traditional public health classification encompasses a broad range of interventions that include routinely used treatments (i.e., tertiary prevention, or the treatment of

Type	Targets	Examples
<i>Universal preventive intervention</i>	An entire population. Desirable for everybody in the eligible population regardless of one’s level of risk for the disease, disorder, or adverse outcome	E.g. Fluoridation of drinking water, fortification of food products, seat belt laws, media campaigns, and drinking age limits to prevent substance abuse
<i>Selective preventive intervention</i>	Members of a population with higher than average risk factors. A risk group may be identified based on psychological, biological, or social risk factors	Lifestyle modification and pharmacological management of hyperlipidemia, group-based psychological treatments for children of depressed parents
<i>Indicated preventive intervention</i>	Members of a population with subsyndromal symptoms of a disorder (or diagnosed with another associated disorder). High-risk individuals may be identified as having minimal but detectable signs or symptoms foreshadowing a disease or disorder—or a biological marker indicating a predisposition to a disorder—although diagnostic criteria for the illness are not yet met	Detection and targeted treatment of the metabolic syndrome, early intervention in psychotic prodrome

established disease to reduce disability). However, the newer IOM classification focuses prevention on interventions occurring before the onset of a formal DSM/ICD disorder. In fact, the IOM report specifically

states that the term prevention is reserved for those interventions that occur before the onset of the disorder, whereas treatment refers to interventions for individuals who meet or are close to meeting diagnostic criteria.

Risk and protective factors

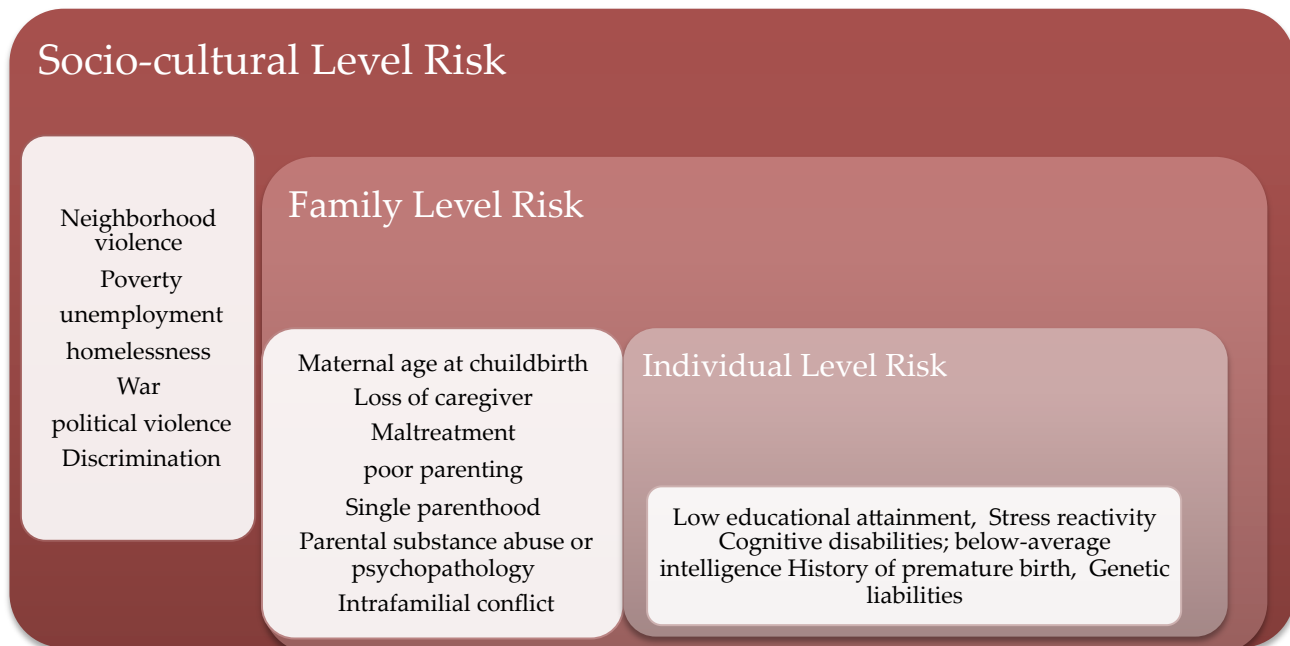
- ★ Risk factors predate the associated disorder; while some are easily identifiable and are malleable via a preventive intervention, some may not be changeable.
 - Biological risk factors include genetic vulnerability, adverse prenatal event (traumatic, toxic, infectious)
 - Psychological/Psychosocial risk factors include family discord, parenting skill deficits
 - Social/Environmental risk factors include availability of drugs and firearms, extreme economic and social deprivation etc.
- ★ Protective factors predate the associated disorder; while some are easily identifiable and are promotable via a preventive intervention, some may not be. Examples include support from caring adults, good school performance, conflict resolution skills, and positive role models clear and consistent discipline in the family.

PREVENTION PARADOX

Universal prevention approaches achieve maximum benefit in practice by reducing disease burden at a population level, but they offer only a small benefit to the individuals who receive such intervention.

At population level, high-risk individuals who will get maximum 'individual' benefit from prevention approaches contribute only to a small proportion of disease burden.

This was first described by Geoffrey Rose in 1981



Socio-cultural Protective Factors

High-quality
educational
opportunities
Socioeconomic
advantage
Supportive
relationships with peers
or nonfamilial adults

Family Protective Factors

Warm and
supportive
parenting or family
relationships
Mentors or other
adult role models

Individual Protective Factors

Cognitive abilities; above-average
intelligence Positive self-perceptions, self-
esteem, Sense of humor Self-regulation
skills (impulse control, coping, emotion
regulation)

2. Physical treatments in psychiatry

Electroconvulsive therapy (ECT)

In 1938, Lucio Cereletti and Ugo Bini conducted the first electrical induction of a series of seizures in a catatonic patient and produced a successful treatment response.

Indications

Condition	Indication
Depressive illness	ECT is considered as a first line treatment for (1) emergency treatment of depression where a rapid definitive response is needed (2) treatment resistant depression and who has responded to ECT in a previous episode of illness. ECT may be the treatment of choice when the depressive illness is associated with; a. Life threatening situation because of refusal of food and fluids b. High suicide risk c. Stupor d. Marked psychomotor retardation e. Depressive delusions and hallucinations (Psychotic depression) f. Patients who are pregnant, if there is concern about the teratogenic effects of antidepressants and antipsychotics. It may be considered as second or third line treatment of depressive illness not responding to antidepressant drugs
Mania	May be considered for the treatment of mania when associated with a. Life threatening physical exhaustion b. Prolonged and severe mania with lack of response to all other appropriate drug treatments
Schizophrenia	May be considered as a fourth line option for treatment resistant schizophrenia after treatment with 2 antipsychotic drugs and then clozapine has proved ineffective, though it is rarely used for this purpose in current practice.
Catatonia	May be indicated in patients with catatonia where treatment with a benzodiazepine (usually lorazepam) has proved ineffective
Parkinson's disease	As an adjunctive treatment for motor, psychotic and affective symptoms in Parkinson's disease with severe disability despite medical treatment.
Others	Neuroleptic malignant syndrome Intractable seizure disorders (acts to raise seizure threshold)

Contraindications

There are no absolute contraindications.

The relative contraindications include the following;

- ❖ Acute respiratory infection
- ❖ A history of recent myocardial infarction (within 3 months and depending on severity)
- ❖ Uncontrolled cardiac failure
- ❖ Cardiac arrhythmias
- ❖ Recent cerebrovascular accident (within 1 month and depending on severity)
- ❖ Raised intracranial pressure
- ❖ Untreated Cerebral aneurysm
- ❖ Intracerebral haemorrhage
- ❖ Untreated Pheochromocytoma
- ❖ Unstable major fracture
- ❖ Deep vein thrombosis- until anticoagulated (to reduce risk of pulmonary embolism)
- ❖ Acute/impending retinal detachment
- ❖ High anesthetic risk

Summary of the UK ECT review group meta-analysis:

1. Real ECT was significantly more effective than simulated ECT. The mean difference in the HDRS is 9.7 (5.7 - 13.5) in favour of real ECT.
2. Premature discontinuation happened equally in both real ECT and simulated ECT groups.
3. Treatment with ECT (both B/L and U/L) was significantly more effective than pharmacotherapy (various drugs), translating to a mean difference of 5.2 points (1.4 - 8.9) on the HDRS.
4. Premature discontinuation was significantly lesser in ECT than pharmacotherapy groups.
5. Bilateral ECT was more effective than unipolar ECT. But greater cognitive impairment was seen among patients treated with bilateral ECT.
6. No difference in efficacy between twice a week and three times a week ECT, or between once a week and three times a week ECT, was noted.
7. High electrical dose led to a larger effect especially in bilateral ECT, but the effect was not significant. But patients treated with high-dose unilateral ECT took longer to regain orientation.
8. No significant difference in efficacy was seen between brief pulse and sinewave ECT.

Rose (2003; BMJ) analysed 35 studies of patient views about ECT; she reported that nearly 1/3rd had significant memory loss. Later in 2005 she reported that nearly 50% of those who undergo ECT do not

receive sufficient information on ECT. Nearly 1/3rd though consented, felt coerced into ECT treatment. In the years 2006 and 2007 the rates of ECT usage were only 0.82 and 0.88 patients per 10 000 total population. This is approximately a third less than the rate in 2005, and three-quarters less than the rate in 1993 *The British Journal of Psychiatry* (2008) 192: 476.

Side effects

- ❖ **Early side effects**- headache (48%), temporary confusion (27%), nausea/vomiting (9%), muscular aches (5%).
- ❖ The **memory deficits** due to ECT can be either anterograde deficits (inability to learn new events/information) or retrograde (inability to recall previously learnt information). These deficits are worse during the treatment period, especially when bilateral ECT is used.
- ❖ With increasing time after the last administration of ECT, a substantial reduction occurs in the extent of **retrograde amnesia** but this is very gradual and a number of patients may be left with incomplete recovery of the lost memories. Older memories for personal events are more likely to be recovered.
- ❖ **Anterograde amnesia** resolves more rapidly after ECT is stopped and patients rarely experience any difficulties in learning new information especially after achieving remission of depressive symptoms.
- ❖ There is no credible evidence that ECT causes any kind of **structural brain damage**. ECT does not affect **executive function** or IQ.
- ❖ **Mortality**: It is no greater than for general anesthesia in minor surgery (2:100000). The risks are related to anesthetic procedures and are greatest in patients with cardiovascular disease. When death occurs it is usually due to ventricular fibrillation or myocardial infarction.

Limitations: ECT has time-limited action and with poor durability (i.e. after a response, most patients will relapse). The effect tends to dissipate after a couple of weeks, hence the need for follow up medication or maintenance treatment. **51.1%** of responders relapse by 12 months, **37.7%** relapsing within the first 6 months, despite continued pharmacotherapy or continuation ECT. In general, the use of antidepressants halves the risk of relapse in the first 6 months (number needed to treat=3.3). The largest evidence base for efficacy in post-ECT relapse prevention exists for tricyclic antidepressants. Published evidence is limited or non-existent for other commonly used newer antidepressants or augmentation strategies.

The practical aspects of ECT administration

ECT work up: Obtain full medical history and list of current medications; perform full physical examination including routine blood tests- FBC, U&Es, any other relevant investigations. In some, pre-ECT Chest X-ray or/and ECG may be needed.

Treatment plan: The optimal frequency is twice weekly administration with 6-12 treatments in total for

one course. If no clinical improvement at all is seen over the first six bilateral treatments, then it is highly unlikely that more treatments will bring about either significant clinical improvement or eventual recovery (Segman et al 1995). Memory should be assessed after each treatment. Significant cognitive impairment should lead to a reappraisal of the electrical dose and electrode placement.

Electrode placement:

- ★ The electrodes are applied to both temples in bilateral ECT and to the temple and to the parietal surface in unilateral frontal ECT
- ★ In bilateral ECT, the centre of the electrode should be 4 cms above and perpendicular to, the midpoint of a line between the lateral angle of the eye and external auditory meatus
- ★ In unilateral ECT, the centre of one electrode is in the same position as in bilateral ECT. The other electrode is applied over the parietal surface of the scalp over the non dominant hemisphere, close to the vertex of the skull

Effective treatment

- ★ Generalised cerebral seizure activity is a necessary ingredient for clinical efficacy but how the seizure is induced has substantial bearing on the outcome.
- ★ EEG monitoring is the gold standard with a typical ECT induction EEG having 4 phases: build up of energies, spike and wave activity, trains of lower voltage slow waves and an abrupt end to activity followed by electrical silence. This will usually last 35-130 seconds
- ★ The effective treatment is defined as a motor seizure lasting at least 20 seconds (from end of ECT dose to end of observable motor activity)

When to use bilateral ECT

- ★ Where speed and completeness of response have priority
- ★ Where unilateral ECT have failed
- ★ Where previous ECT has produced a good response without undue memory impairment
- ★ Where determining cerebral dominance is difficult

When to use unilateral ECT

MAINTENANCE ECT

Maintenance ECT should be considered when

- ❖ *The index episode of illness responded well to ECT*
- ❖ *There is an early relapse despite adequate continuation drug treatment*
- ❖ *Inability to tolerate continuation drug treatment*
- ❖ *The patient's attitude and circumstances are conducive to safe administration.*

- ★ Where speed of response is less important
- ★ Where there has been a previous good response to ECT
- ★ Where minimising memory impairment is particularly important

Psychiatric drugs and ECT

- ★ The class of drugs that raise seizure threshold include benzodiazepines, barbiturates and anticonvulsants
- ★ The class of drugs that lower seizure threshold include antipsychotics, antidepressants and lithium
- ★ Clozapine should be suspended 24 hours before ECT
- ★ Moclobemide should be suspended 24 hours before ECT
- ★ Lithium may be best avoided as it may increase cognitive side effects and increase likelihood of neurotoxic effects of lithium

Transcranial magnetic stimulation (TMS)

- ★ In 1985, Anthony Barker developed the first device that used magnetic field to produce stimulation of a targeted brain region.
- ★ TMS is rapidly evolving as a treatment option for depression. It is probably the most studied of all somatic treatments for depression.
- ★ It involves the application of magnetic pulses on the scalp surface, which creates an electrical activity that stimulates neurons in cortical surface in line with Faraday's principle of electromagnetic induction.
- ★ Single pulse TMS has been found useful for the treatment of migraine. In depression, repetitive pulses of TMS are used (repetitive TMS or rTMS).
- ★ rTMS was approved by US FDA to be used in the treatment of depression that has failed to respond to a trial of antidepressants. For this indication it has an NNT of 4 (Furukawa, 2014). TMS is now endorsed by many national guidelines including the BAP guidelines of depression.
- ★ To treat depression, rTMS must be applied to either left or right DLPFC (most often the left) for a period of 30-40 minutes a day for at least 4 consecutive weeks.
- ★ TMS does not require anaesthesia and is usually carried out as an outpatient procedure. There is no recovery period required, and the patient can immediately resume activities after a session of TMS. Furthermore, TMS does not induce cognitive side effects such as amnesia – a major reason for its expansive growth in recent times.
- ★ While the effectiveness of ECT is based on inducing a generalised seizure, TMS aims to stimulate focal brain region without inducing a generalised stimulation that results in seizure. rTMS possibly harnesses the inherent plasticity of brain circuits to enable strengthening of connectivity between brain regions that are malfunctioning in depression.
- ★ Outcome studies show that in depressed patients who have failed to respond to an average of 2.5 trials of antidepressants, TMS application can produce 40% response rates that can be sustained beyond 6 months by applying 'rescue' TMS when required (Janicak & Carpenter 2012).

- ★ When compared head to head with ECT, the short-term response rates for ECT is significantly superior especially in the presence of psychotic features (Ren et al., 2014).
- ★ Side effects include discomfort on the site of application, headaches that are transient and do not persist beyond the treatment period (10%) and facial muscular twitching during stimulation (transient). Theoretically, TMS can induce a seizure especially when applied to motor cortex. But in practice, the prefrontal application used in depression rarely induces seizure (no cases reported in 45 RCTs so far)
- ★ TMS has also shown to be effective in resistant auditory hallucinations in psychosis, if applied to the left temporoparietal cortex (close to Wernicke's area). But the size of this effect is smaller than the efficacy rates reported for depression.

Psychosurgery

In 1995, the first 'pre-frontal leucotomy' was carried out by Moniz and Lima. Currently, psychosurgery is only performed in rare cases when all other treatments have failed. Psychosurgical procedures focus on specific brain regions e.g. lobotomies and cingulotomies or their connecting tracts e.g. tractotomies and leukotomies implicated in the pathophysiology of psychiatric illnesses.

Current criteria

- ★ Severe mood disorder or obsessive compulsive disorder that has been resistant to all other appropriately reasonable evidence-based treatments tried in adequate dose for adequate duration
- ★ The patient is competent and provides informed consent for the surgery

Current surgical techniques employ **stereotactic methods** using pre-operative MRI to establish target co-ordinates and a fixed stereotactic frame. Lesions are localised to the orbito-frontal and anterior cingulate loop (the limbic loop), which is strongly implicated in the regulation of mood and emotions. The lesions are produced either by radio-frequency thermocoagulation or gamma radiation (the gamma knife).

The stereotactic procedures used in psychosurgery include

Subcaudate tractotomy

- lesion made beneath the head of each caudate nucleus, in the rostral part of the orbital cortex

Anterior cingulotomy

- bilateral lesions within the cingulate bundles

Limbic leucotomy

- combining subcaudate tractotomy and anterior cingulotomy

Anterior capsulotomy

- bilateral lesions in anterior limb of internal capsule

Adverse effects: Older techniques are associated with severe amotivational syndromes (4%), marked personality change (up to 60%) and epilepsy (up to 15%).

With modern procedures severe adverse effects are less frequent and include

- ★ Headache and nausea after the operation.
- ★ Confusion occurs in 10% of patients and lasts a few days.
- ★ Long-term cognitive impairment does not seem to occur.
- ★ Newer stereotactic techniques report minimal post-operative problems with personality change and social functioning (2-8%) or cognitive functioning. As measured by IQ scores, cognitive abilities improve after surgery probably because of patients' increased ability to attend to cognitive tasks.
- ★ Post operative seizures is seen in < 1% of patients (usually controlled with phenytoin).
- ★ Operative mortality is less than 0.1%.
- ★ Weight gain is reported in 10% of patients.

Outcome: The reports of good outcome are surprisingly high given the treatment resistant nature of the patients receiving surgery.

- ★ When patients are carefully selected, between 50 and 70% have significant improvement with psychosurgery. Fewer than 3% become worse. Continued improvement is often noted from 1 to 2 years after surgery.
- ★ Chronic intractable major depressive disorder and OCD are the two disorders reportedly most responsive to psychosurgery.

- ★ A report by the Royal college of Psychiatrists in 2000 highlights the observation of 'marked improvement rates'- 63% of patients with major depression and 58% of patients with OCD.
- ★ Stereotactic limbic leucotomy and anterior capsulotomy are used for OCD; Stereotactic subcaudate tractotomy is the treatment of choice for mood disorder.

Deep brain stimulation (DBS)

Deep brain stimulation involves the use of fine wire implants in certain brain regions that can be triggered using a subdermal pacemaker device placed on the chest wall. High frequency electrical stimulation can temporarily 'arrest' the activity of the brain region.

Indications: DBS in select brain regions has provided therapeutic benefits for the treatment of movement disorders like Parkinson's disease, essential tremor, Tourette's disorder and dystonia. It has been evaluated for use in major depression, obsessive-compulsive disorder (OCD) and chronic pain (neuropathic and phantom limb pain).

- ★ DBS at the subthalamic nucleus and the internal globus pallidus is endorsed by many guidelines for treatment of Parkinson's disease. It may be as effective as medications, but without motor side effects in many patients.
- ★ DBS of the internal capsule has shown positive effects for OCD.
- ★ DBS can also reduce abnormal movements in patients with Tourette's disorder.

Advantages: DBS is less invasive than ablative surgery. The device can be turned off when not required and the wires can be easily removed without much sequelae.

Side effects: Surgical complications due to DBS include infection (0–15%), intracranial haemorrhage (0–4.5%), stroke (0–2%), lead erosion, lead fracture, lead migration, and death (0–4.4%). Infection remains the most common surgical complication. The rates vary between 0% and 15% per patient and 0% and 9.7% per electrode. The rate of intracerebral hemorrhage related to DBS surgery remains relatively low overall, with rates ranging from 0% to 4.5%. The reported incidence of postoperative seizures has varied considerably from 0% to 4%, although rates have been approximately 2%, with the risk of epilepsy essentially being nil. The frequency of death has ranged from 0 to 4.4%.

The neuropsychiatric side effects include depression, anxiety, mania, impulsivity, impulse control disorders, speech and language difficulties, decreases in various measures of cognitive performance, and postural instability with increased falls. A considerable number of small case series have reported on the risk of depression with DBS, with a recent literature review reporting a postoperative incidence of 1.5%–25%.

Vagus Nerve Stimulation

Vagus Nerve Stimulation (VNS) refers to stimulation of the left cervical vagus nerve. It is available for the treatment of resistant partial-onset seizures in epilepsy.

The basic mechanism of action of VNS is unknown. It may result in changes in serotonin, norepinephrine, GABA, and glutamate neurotransmitters implicated in the pathogenesis of major depression.

3. Community services in psychiatry

Multidisciplinary teams

- ★ A multidisciplinary team is defined as “a group of people with complementary skills who are committed to a common purpose, performance goals, and approach, for which they hold themselves mutually accountable.”
- ★ Sainsbury Centre for Mental Health (1997) listed **key competencies** for multidisciplinary working as follows: (1) assessment (2) treatment and care management (3) collaborative working (4) team management and administration (5) interpersonal skills.
- ★ The concept of competency can be extended to include the ability to apply the necessary knowledge, skills and attitudes to a range of complex and changing settings. This is referred to as **capability**.
- ★ Moss (1994) highlighted several **key functions** for multidisciplinary teams (1) continuing proactive care of those with long-term serious mental health problems (2) Uninterrupted access to information and support, intervention and treatment before and during crises, and (3) an organised response to requests for help from primary care.
- ★ Benefits:
 1. MDTs maximize clinical effectiveness.
 2. They reduce bed use (Dean *et al.*, 1993)
 3. Ensure that patients maintain service contact after discharge from hospital (Ford *et al.*, 1995).
 4. They are cost-effective (Knapp *et al.*, 1994)
 5. They enable the provision of a wide range of services and resources without prolonged referral processes (Campbell *et al.*, 1998)
 6. They enhance continuity of care (Towell and Beardshaw 1991).

(Summarized from *Multidisciplinary Team Working: From Theory to Practice*.
Mental Health Commission of Ireland discussion paper)

Policy on community psychiatric services

National Service Framework for Mental Health (NSF) published by the Department of Health in 1999 was launched as a comprehensive agenda for mental health services with a clear emphasis on community psychiatry. It emphasized several key standards for service development (7 in total).

The **NHS Plan** was proposed in 2000. This provided the targets and funds to realize NSF proposals – in particular to improve intensive community care teams. Three critical targets were the focus of much management action within

NSF STANDARDS

1. Promote mental health, reduce discrimination and exclusion
- 2 and 3. Better primary healthcare
- 4 and 5. Ensure crises care, timely access to secure and safe place, as close to home as possible
6. Ensure carers needs are addressed
7. Public health target on suicide (to be reduced by 1/5th in 2010)

mental health services:

1. 50 early intervention teams to be in place by 2004
2. 335 crisis resolution teams by 2004
3. 220 assertive outreach teams by 2003.

A new mental health strategy for England

was published in **2011**. It proposed 6 shared objectives.

1. More people will have good mental health (starting well, developing well, working well, living well and ageing well).
2. More people with mental health problems will recover (improving quality of life)
3. More people with mental health problems will have good physical health
4. More people will have a positive experience of care and support (offer timely evidence based interventions)
5. Fewer people will suffer avoidable harm. (improving confidence on services).
6. Fewer people will experience stigma and discrimination. (improve public understanding)

PRINCIPLES OF ACT MODEL

1. *Continuous service – both in terms of time service provision and functional continuity*
2. *Staff ratio 1 to 10-15 patients*
3. *Most work being done in the community*
4. *Team involvement for each case – not individual case managers*
5. *Individualized treatments with no time constraints*
6. *Direct assistance and service delivery without commissioning whenever possible*
7. *Primary outcome is functional improvement*
8. *Bulk care (core service) is provided by the team – no brokerage*
9. ***Titrated input** – as and when needed basis.*
10. ***Backing off** service provision in times of need - not completely withdrawing if patient refuses to engage*

Models of providing community care

Brokerage model: Here the main worker acts as a broker coordinating various services while not actively providing any input.

Case management: In case management model a single professional is responsible for long term supportive care for all aspects. This model stresses continuity of care similar to key worker model proposed in Building Bridges document by Dept. of Health 1995. The clinical care includes engagement, assessment, planning interventions and delivering them directly followed by monitoring effectiveness.

Assertive community treatment (ACT): Stein & Test 1980 evaluated 'training in community living'. For them the main issue was transfer of learning in social skills training in real life when patients move from inpatient units or rehab wards to community. They saw the community care provided then as leading the patient to a 'tenuous community adjustment on the brink of hospitalization'. National service framework and NHS plan endorsed the need for assertive community treatment.

UK 700 study reported that caseload is the most important predictor of outcome of an ACT service. ACT approach is shown to reduce admissions, reduce acute presentations at the A&E and increase compliance with secondary care. Cochrane review showed that no clear advantage could be demonstrated between ACT and intensive community management.

Intensive case management

- Proposed for hard to engage patients
- Follows principles of ACT but individual case loads to reduce the redundant time spent discussing cases
- The main drawback was heavy staff burn out and responsibility

Personal strengths model

- Also called development – acquisition model.
- Patient is the primary director of the process
- It acknowledges that the patient is able to grow and change inherently
- Therapeutic relationship is given primary importance
- No goals are imposed on patient – community is seen as valuable resource.

Rehabilitation model

- Similar to personal strengths model
- Assessment based comprehensive rehabilitation plan is drawn
- Patient directed process but deficit focus is retained
- Increased patient autonomy and independence is stressed.

Community mental health teams (CMHTs)

- ★ CMHTs are known as the **basic building blocks** for community psychiatric services. CMHTs generally comprise of psychiatrists, community psychiatric nurses, social workers, psychologists and occupational therapists.
- ★ The most important gain when compared to outpatient services/regional hospitalised care is the **continuity of care**.
- ★ Geographically limited CMHTs can improve patients' engagement with services, promote mental health user satisfaction, reduces unmet needs and improves treatment adherence, although the gain in social function is questionable
- ★ **Case management** and **keyworking** is the predominant approach to deliver care within CMHTs. According to this model, keyworkers come from any of the various disciplines and act as the prime therapist who coordinates and leads the care plan and serve as the patient's and carers' main point of access to the team. Case Management can be described as described as the 'coordination, integration and allocation of individualised care within limited resources' (Thorncroft, 1991). While its utility in in-patient services is questionable, it has a good evidence based support for improving continuity of care, satisfaction and social function in patients benefitting from CMHTs. Case management is less intensive and more non-specific than assertive community treatment.

Care programme approach

- ★ The origins of the care programme approach (CPA) can be traced back to the Spokes Inquiry into the care of Sharon Campbell who killed her social worker. (DHSS, 1988). In 1991 an active 'Care programme approach' was introduced.
- ★ The basic requirements of the CPA include:
 1. A thorough assessment of health and social needs
 2. Each service user must have a written care plan
 3. Service user must be involved in drafting their care plan
 4. This plan must be reviewed regularly or as necessary
 5. A named mental health worker must coordinate care delivery.
- ★ Levels of CPA have been simplified in 1999 to:
 - **Enhanced** – in practice, for those whose care needs are best served by regular multi-disciplinary review meetings
 - **Standard** – where such meetings are unnecessary.
- ★ Modernizing CPA has suggested other changes including
 1. Integrate the CPA with care management to form a single care coordination approach
 2. Appoint a lead officer to work across all agencies
 3. Apply CPA as a framework for mental health care delivery and not simply as an after-care arrangement;
 4. Abolish supervision registers
 5. Change the name of the key worker to 'care coordinator'

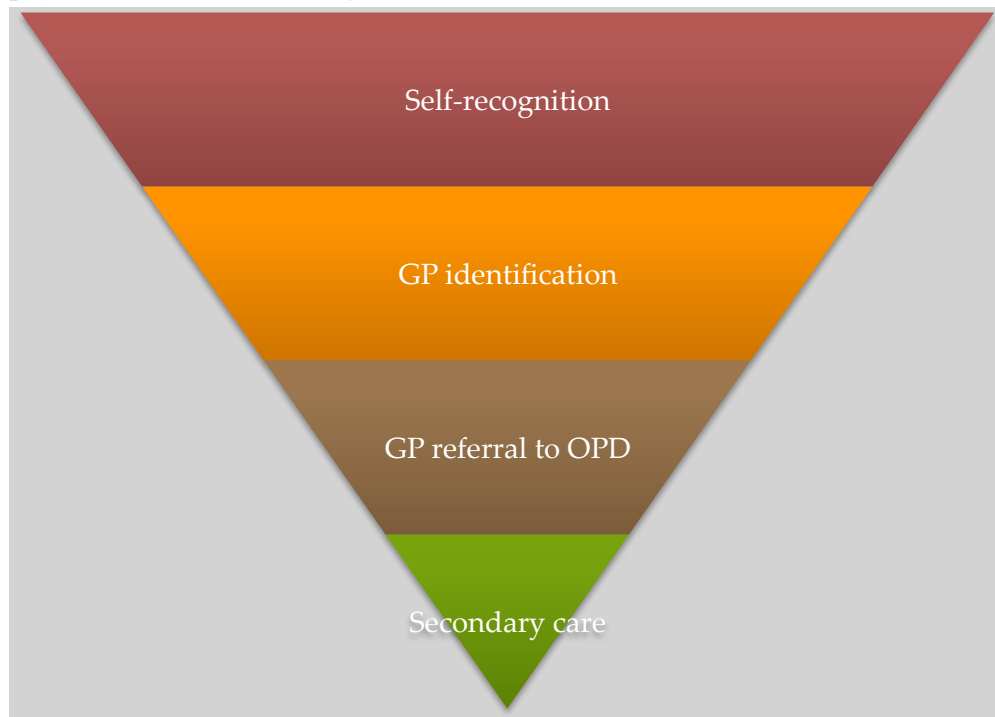
Service Utilisation

From pooled analysis of WMH surveys, median delays among cases eventually making contact was estimated from 3.0 to 30.0 years for anxiety disorders, from 1.0 to 14.0 years for mood disorders, and from 6.0 to 18.0 years for substance use disorders. Failure and delays in treatment seeking were generally greater in developing countries, older cohorts, men, and cases with earlier ages of onset. These results show that failure and delays in initial help seeking are pervasive problems worldwide

Pathways to care:

Also called as **filter model**, this was developed by Goldberg and Huxley, to account for how mental illness interacts with the healthcare system. Five levels of mental illness occurrence were described: The community, the primary care attendees, the diagnosed primary care attendees (in whom the mental illness has been recognised), the level of psychiatrist and that at the level of psychiatric inpatient care.

Four filters explain the decreasing numbers of cases when going from the general population to inpatient psychiatric care (see the figure attached)



1. At the level of the patient himself or herself (recognition)
2. At the level of the general practitioner (recognition, decision to treat, decision to refer),
3. At the outpatient level of the mental healthcare system and
4. At the inpatient admission level.

Early Intervention Services

Early intervention refers to an attempt to change the trajectory of severe mental illnesses (especially psychosis) and improve long-term prognosis by intervening early, in some cases even before full-blown symptoms are apparent.

The concept of the 'critical period' was proposed by Birchwood to refer to the the first 3–5 years from the onset of psychosis wherein psychosocial plasticity is higher and the greatest impact can be made if interventions are instituted.

One of the key challenges in indicated prevention in schizophrenia is to determine which signs and symptoms are the precursors to the full syndrome, i.e. identification of a preschizophrenic state. Various terms have been used to describe the prodrome of schizophrenia. The term prodrome can be used only retrospectively after one develops the full-blown illness. '**Prodrome**' carries a sense of inevitable progression. '**Premorbid state** or '**at risk mental state**' has been preferred by some.

Early Intervention in psychosis includes **both early identification and treatment**. The tenets of early intervention include:

1. Reducing duration of untreated psychosis (DUP)
2. Promoting recovery

3. Minimizing secondary morbidity and mortality
4. Reducing psychosocial damage
5. Engagement with the patient at early stages to facilitate longer treatment
6. Reducing comorbidities such as substance use

Criteria for early identification

- ★ **PACE – UHR** (Personal assistance and Crisis Evaluation service – Ultra High Risk) criteria:
 - Uses ‘close in’ strategy
 - Specificity prioritised over sensitivity
 - Uses age 14 to 30; considers experiencing attenuated positive symptoms (APS) or episodes of frank psychosis (BLIPS- Brief limited intermittent psychotic symptoms); having schizotypal personality or having family history of schizophrenia.
 - Prevention through Risk Identification, Management and Education (PRIME) study at Yale, Early Identification and Intervention Evaluation (EDIE) Clinic at Manchester etc have adapted the PACE UHR.
- ★ **Basic symptoms:** German Psychopathologists have known for long what are termed as basic symptoms to predate onset of schizophrenia; these include cognitive, affective and social disturbances part from depressed mood, irritability, aggression etc. Bonn Scale for Assessment of Basic Symptoms predicts the conversion from basic symptom to schizophrenia in 78% individuals.
- ★ **Various other criteria are also in use** (e.g. Hillside criteria)

Evidence for early intervention

1. There is a clear evidence favouring EI in Psychosis from 1-, 2- and 5-year follow up of randomized EI teams (OPUS trial carried out in Denmark, Lambeth Early Onset trial (LEO)) and from first episode cohorts evaluated by PEPP (London, Ontario) and TIPS (Norway). These results suggest that the initial gain from Early Intervention approach may not be sustained if the EI services are discontinued after the initial 2 years.
2. CBT may have a role in reducing progression from at risk state to full blown schizophrenia (Morrison, 2004). The SoCRATES study (Study of Cognitive Reality Alignment Therapy in Early Schizophrenia) compared CBT with supportive counselling for a first- or second-episode of schizophrenia wherein at 18-month follow-up (Tarrier et al., 2004), the addition of both CBT and supportive counselling showed a significant improvement.
3. PRIME study used **olanzapine** in a low dose to prevent progression with favourable results.
4. EPPIC from Melbourne (McGorry et al) reported that a combination of CBT and **risperidone** could reduce the conversion rate – at 6 months; though longer follow-up suggested that a ‘delay’ not ‘prevention’ effect was possible. Warner reappraised the results of EPPIC report and found out that even one additional case of schizophrenia would have nullified the much-glorified results.
5. The Lambeth Early Onset (LEO) study compared specialised care (low dose atypical anti-psychotics, CBT using manualised protocols, family counselling and vocational strategies) based on a model of assertive outreach with standard care. The relapse rates were lower with specialised care.

Obstacles for early intervention

1. It is unclear whether 'earlier use' of available interventions in itself is really effective. Natural course of the illness itself may be one of self-recovery in some patients as exemplified by concepts such as Soteria project. The exposure to medications for this subgroup may essentially be only toxic.
2. With low specificity of screening instrument high rates of false positivity and unnecessary treatment of those falsely positive will take place.
3. Any intervention at prodromal stages could only be delaying not completely preventing the psychosis.

Duration of Untreated Psychosis (DUP)

- Wyatt identified DUP or length of untreated manifest illness as a prognostic marker. He also suggested that untreated psychosis may be neurotoxic.
- DUP is thought of as a critical or sensitive period when correct identification and intervention can reduce later disability.
- It is difficult to define duration of untreated psychosis. One struggles to identify where to start and stop measuring the interval.
- If 'untreated' psychosis refers to lack of pharmacological treatment, then before 1960, DUP was equal to length of lifetime illness for most patients. Recovery rates did increase in the mid-20th century but antipsychotics have not made a huge difference to the proportion of patients in remission in long-term follow-up studies (Hegarty et al, 1994). Hence DUP may not be a useful measure.
- Length of DUP does not correlate directly with outcome in most secondary research evidences (Rankin et al 2005). In developing countries where DUP is in terms of nearly 10 to 15 years in some cases, the outcome is not grossly different once identified and treated.
- DUP is confounded by personality and/or illness-related variables, with a combination of insidious onset, negative symptoms and premorbid dysfunction contributing to delayed help-seeking, delayed initiation of treatment and poorer outcome.
- In the UK, the Department of Health has set a national early intervention target of reducing DUP to a service median of 3 months and an individual maximum of 6 months. Median DUP in well-established community services appears to be relatively low: 52 days for schizophrenia in Nottingham, UK, in one study (Singh et al, 2005a).

The concept of neurotoxicity

EI services were initially supported on the basis of the idea of neurotoxicity. It is hard to accept that psychosis can be neurotoxic. This is because

1. No cell death is evident; no gliosis is evident with cumulative episodes of psychosis
2. Cognitive impairment must be cumulative if each episode is neurotoxicity- this is not the case so far known.
3. While measuring prognostic factors, number of episodes one has seems less important than age of onset of psychotic illness.

4. Rehabilitation service

Most of the rehabilitation psychiatry is in reference with schizophrenia literature, as psychosis is the most severe of psychiatric disorders with respect to the degree of functional deficits.

Recovery: Different stakeholders variously define 'Recovery'. In medical definition recovery stands for complete cure as an outcome. Remission is necessary but not sufficient for recovery. Remission is defined as reduction in core signs and symptoms to an extent that they no longer interfere with behaviour and not justifiable for making an initial diagnosis at that point (Recovery in schizophrenia working group). Meanwhile recovery is defined as being relatively free of any psychopathology.

Vermont longitudinal study was the first study that challenged the pessimism about schizophrenia recovery. Soon more studies followed suggesting substantial number of patients recover from 'dementia praecox'. International study of Schizophrenia found that nearly 48% of patients recovered at 15 years and 25 years follow-up by loose criteria – using strict criteria 38% showed recovery.

The consumer definition of recovery concentrates on the process rather than outcome. Jacobson & Greenley in their recovery model identified internal and external conditions for recovery.

Internal conditions	External conditions
Hope of recovery	Valuing human rights
Healing as primary aim	Services oriented towards recovery
Empowerment	Appraising positive value of healing
Connection with other services and patients	

Vocational rehabilitation *(From Morris & Lloyd 2004)*

Skills training/ prevocational training

- It is the core of most current psychiatric rehabilitation units in UK.
- Skills development improves rehabilitation results, diminishes the demand for clinical services and increases the likelihood of gaining employment
- May include NVQ, vocational practice, sheltered workshops etc.

Transitional employment (TE) programmes

- Emerged from the clubhouse model developed by Fountain House in 1948
- Transitional employment is a time-limited, supported work experience
- Transitional employment programmes typically operate out of clubhouses.
- Clubhouses provide members with a meaningful day,

CHARACTERS OF RECOVERY ORIENTED SERVICES

1. *Self directed*
2. *Person centred*
3. *Peer supported*
4. *Empowerment focused*
5. *Holistic*
6. *Non linear (not stepwise)*
7. *Strengths based*
8. *Respect driven*
9. *Responsibility shared*
10. *Instilling hope*

educational opportunities, in-house vocational training and social/recreational options.

- The clubhouse owns some employment positions which are filled consecutively by clubhouse members, usually over a 6-month period and frequently in a part-time capacity
- The person may then graduate to competitive employment in a part-time or full-time capacity.

Sheltered employment

- Reserved entry to jobs – e.g. quota for the mentally ill etc.
- Segregated workforce
- Often not commercially viable e.g. Remploy in UK

Supported employment (SE) programmes

- Became prominent in the 1990s.
- The individual is placed in a full or part-time job supported by an employment consultant
- There is usually a minimum of prevocational training.
- 'Place and train' – not 'train and place'; this makes transfer of learning easier (Drake & Becker model)
- The major aim is to get the individual into a job and then support the individual as they perform their duties.
- A number of techniques to facilitate this process, - 'choose-get-keep', 'job coach', assertive community treatment and individual placement and support
- Characteristics of successful SE programmes
 1. Commitment to a competitive employment goal
 2. Rapid job search and placement;
 3. Jobs selected on the basis of individual preference and the skills and experience of the person;
 4. Follow-up employment consultant support and case management is maintained indefinitely
 5. Close integration of SE programmes with mental health teams

Improving compliance to treatment (from Tacchi & Scott, 2005)

★ **Treatment gap**

- Refers to gap between effectiveness and efficacy of treatment;
- Causes include
 - Drug factors – side effects, idiosyncratic inefficiency
 - Patient factors such as co morbid illness,

<i>Compliance</i>	<i>Adherence</i>	<i>Concordance</i>
• Has a sense of patient being passive • Paternal connotation • Seems to be one-dimensional on 'medication taking behaviour'. • Non compliant is meant as 'bad behaviour'	• Has more dynamic sense • Implies a spectrum of behaviours • But concentrates on outcome – 'adhered to or not'.	• 'Expert patient' philosophy • Differences between doctor patient are legitimized • Negotiation is implied as a key factor • Gives respect to patient preference • Stresses on process than outcome.

health beliefs, stigma, influence of significant others

- Treatment gap in mental health care is very large – Kohn 2004 WHO bulletin reported from community epidemiological studies that for psychosis worldwide 32% are non treated; in Europe this falls to 18%; in Europe 40% with bipolar are untreated; >50% with depression and anxiety are untreated.
- Non-adherence is the single most important factor increasing risk of hospitalization in the severe mentally ill (Weiden 2004, California Medicaid data).
- Haynes in 2001 Cochrane review reported that improving adherence can have greater impact on population health than any other specific health intervention.
- The median continuous use of lithium is estimated to be only 76 days in those with bipolar disorder.

★ **Measuring compliance**

- Subjective measures: both patients and physicians overestimate actual rates of compliance.
- Objective measures: include pill counts, cumulative possession ratio (days treatment received/days eligible for treatment), serum levels, electronic bottle 'pop' recordings (MEMS – medication event monitoring system).

★ **Myths about adherence**

- a. Non-adherence is specific to mental illness: In 2003 WHO released a document adherence to long-term therapies: evidence for action. This highlights 30-40% of those with chronic medical illnesses such as diabetes does not adhere to treatment.
- b. Adherence is all or none: It varies from complete nonadherence to non-satisfactory, partial, satisfactory and full adherence.
- c. New drugs have less side-effects and so change adherence rates drastically: Nonadherence due to side effects may reduce but overall rates remain much unchanged.

5. Medicolegal issues

Psychiatry & human rights

A summary of Human Right Articles that are relevant to mental health in accordance with **European Convention of Human Rights (ECHR)** is highlighted in the figure below.

Most common human rights violations experienced by the mentally ill: (Drew et al., 2011)

1. Exclusion, marginalisation, and discrimination in the community
2. Denial or restriction of employment rights and educational opportunities
3. Physical abuse/violence
4. Inability to access effective mental health services
5. Sexual abuse/violence
6. Arbitrary detention
7. Denial of opportunities for marriage/right to found a family
8. Lack of means to enable people to live independently in the community
9. Denial of access to general health/medical services
10. Financial exploitation

People with mental illnesses are often restricted from exercising many civil rights. In a survey of such issues from many low-income and middle-income countries, Drew et al. (2011) noted that the right to marry and have children is often denied on the grounds of mental illness. Marriage legislation in many countries endorse the view that being of “unsound mind” or having a long-term mental health condition can be grounds divorce.

Major strategies for improving the human rights in mental health: (Drew et al., 2011)

- ★ Running public-awareness and anti-stigma campaigns and educating about the rights of the mentally ill
- ★ Providing better training of mental health professionals and supporting community mental health services
- ★ Promoting the empowerment, rehabilitation, and participation of people with mental and psychosocial disabilities in their communities
- ★ Implementing effective and humane laws and policies to protect and promote the human rights
- ★ Supporting patient organisations
- ★ Monitoring and assessment of human rights of the mentally ill
- ★ Integrating mental health into overall health and development policies

Right to vote: Hospitalised patients retain their right to vote as long as they are registered on the electoral list. However, if detained under criminal sections under the direction of Courts or transferred from prison, the right to vote is restricted (this is currently under review in the UK).

Article 2: Right to life

- State has a duty to protect lives. All deaths occurring in state detention must be investigated, including suicides. This applies to both formal and informal patients.

Article 3: Prohibition of torture

- Torture, inhuman or degrading treatment cannot be justified in any circumstance
- Provision of prescribed medical treatment itself will not breach article 3 even if it causes intolerable side-effects
- Issues most likely arise with seclusion and detentions.

Article 5: Right to liberty

- Detention for treating mental disorder is acknowledged by ECHR.
- Issues most likely arise with delays in tribunal reviews, detentions that does not comply with MHA 1983, MCA 2005 and DOLS safeguards, prolonged restraint or seclusion and delayed discharges.

Article 6: Right to a fair hearing

- Cannot be interfered in any circumstance
- Applied to mental health review tribunals and Court of Protection.

Article 8: Right to private/family life

- Issues arise in relation to permitting family visits, home leaves, restricting correspondences prohibiting activities such as smoking in care 'homes'.

Article 9: Freedom of thought & religion

- This is a qualified right that can be interfered with in certain circumstances such as to protect public safety, public order and freedom of others.
- Issues may arise if there is a failure to provide a place of worship, right type of food and when asked to stay in mixed wards.

Article 10: Freedom of expression

- This can be restricted in the interest of national security, territorial integrity and public safety.
- Proceedings from MHA tribunals and COP are usually private

Article 12: Right to Marry

- Mentally ill patients detained under section 2 of MHA may not be able to enter a marriage contract

Article 14: Right not to be discriminated against

- Includes race, religion, nationality, disability and gender

Driving

- ★ The DVLA is legally responsible for deciding if a person is medically fit or unfit to drive. But it is the duty of psychiatrists/doctors to inform patients how their conditions could impair driving. We must encourage patients to report to DVLA on their own accord; patients can also be encouraged to seek a second opinion, if they disagree. But if a patient continues to drive when not fit the doctor can break confidentiality and involve authorities that have the power and ability to stop the driver. We must inform the patient of decision to override confidentiality before informing the DVLA.
- ★ For Group 1 (car, motorcycle) drivers with acute psychosis of any cause, hypomania/mania, schizophrenia or other chronic psychosis, driving must cease during the acute illness. Relicensing can be considered when all of the following conditions can be satisfied;
 - The patient has remained well and stable for at least 3 months
 - Is compliant with treatment
 - Is free from adverse effects of medication which would impair driving
 - Subject to a specialist favorable report
 - Regained insight in case of bipolar mania or hypomania
- ★ For group 2 (HGV) drivers driving should cease pending the outcome of medical enquiry. The person must be well and stable for a minimum of three years with insight into their condition before driving can be resumed.
- ★ For group 1 drivers with severe anxiety or depression, driving should cease pending the outcome of medical enquiry. A period of stability depending upon the circumstances will be required before driving can be resumed. This is not necessary for mild depression or anxiety seen in primary care. For Group 2 drivers with severe anxiety or depression, driving may be permitted only when the person is well and stable for a period of six months
- ★ For patients with dementia, license in early stages is subject to annual review for group 1 drivers; but group 2 drivers will get license revoked.

Capacity assessment

Consent

- ★ Consent requires 1. Information 2. Competency 3. Autonomy
- ★ Competence + information → understanding
 - Understanding + autonomy → decision
 - Decision + communication → consent
- ★ A competent adult patient can refuse any treatment (including life saving measures). If the treatment is forced, it will constitute a battery.
- ★ A competent adult patient must be given information about medical procedures. If not it will constitute negligence.
- ★ For incompetent adult patients, the doctor should act in accord with the Mental Capacity Act.

- ★ Incapacity and the remedial actions to be taken are decided in accordance with Mental Capacity Act 2005. Steps include checking for a previously appointed attorney, advanced directives and considering substituted judgment to guide the best interest decision. Relatives can help decide the best interests, but cannot give or withhold consent on the behalf of an incompetent person, unless legally sanctioned as a health attorney.
- ★ Children (less than 18) should not be allowed to come to serious harm on the grounds of refused consent from the minor or parents in case of necessary and urgent treatments.
- ★ Patients between 16-17 are presumed to have capacity to consent unless shown the contrary.
- ★ A 'Gillick competent' child can give consent, but if he/she refuses, parents or court can override the refusal. If not Gillick competent, a parent can give consent, acting in the minor's best interests.
- ★ There are some situations in which explicit consent is not needed. This would include **implied consent** such as when a patient holds out his arm to have his blood pressure measured (consent by consultation).

MEDICAL NEGLIGENCE

It is the most common reason for doctors to attend court. These cases usually arise from the dereliction of duty directly causing damage to the plaintiff. (4Ds).

*In the UK, the **Bolam test** is often used in such cases. Accordingly, a doctor is required to exercise the ordinary skills of a competent practitioner in the field (profession-based standard).*

Capacity

- ★ By default it is presumed that everyone has capacity to make decisions. Imprudent decision itself is not sufficient to suspect incapacity.
- ★ Capacity is not global but task specific.
- ★ Testing comprehension and retention, ability to analyze the information and weigh up consequences, and the ability to communicate the decision are required to assess one's capacity.
- ★ The MacArthur Treatment Competence Study was designed in 1988 to study the decision-making capacities of people who are hospitalized with mental illness. A tool was developed for this purpose which identified four legally-relevant abilities - **Choosing**: abilities to state a choice, **Understanding**: to understand relevant information, **Appreciating**: to appreciate the nature of one's own situation, and **Reasoning**: to reason with information.
- ★ Several versions of the MacArthur Competence Assessment Tool (MacCAT) are now available. MacCAT-CR for clinical research, MacCAT-T for treatment decisions, MacCAT-CA for criminal adjudication
- ★ Hoge et al. developed and validated the MacArthur Structured Assessment of the Competence of Criminal Defendants (MacSAC- CD) to assess a defendant's fitness to plead based on legal theory of competence.

Confidentiality:

Exceptions include (Please refer to GMC guidelines for more details)

1. Court order

2. Statutory requirement to aid legal proceedings: e.g. Misuse of Drugs, Road Traffic Act, Police and Criminal Evidence Act, Terrorism Prevention etc.
3. Veneral Diseases Regulation – to prevent public hazard for communicable diseases

Otherwise, disclosure can be done

1. With consent
2. In public interest
3. Without identification
4. On a need to know basis

Tarasoff case in the USA highlighted the importance of the **duty to warn** third parties when a doctor comes to know of important information that can endanger the third party. This is not legislated in England & Wales.

Use of seclusion

- ★ **Seclusion** refers to the involuntary confinement of a patient within a physical space (a room or any other area) from which he/she is physically prevented from moving freely.
- ★ In psychiatric practice, seclusion is used in inpatient settings to manage violent or self-destructive behaviours.
- ★ **Restraint** is defined as the use of a physical, manual, material or mechanical method to immobilize or reduce the ability of a patient to move his body (or body parts) freely.
- ★ **Pharmacological restraint** is defined as the use of a drug that is not a part of the standard treatment or dosage for the patient's condition in order to achieve a restriction of the patient's behaviour or freedom of movement.
- ★ Both seclusion and restraint must be discontinued as soon as it is practically possible. These practices are often perceived as a form of social control and are frequently traumatic to patients. But if carried out within the lawful practices, it is justifiable to seclude a patient without necessarily contravening his/her Human Rights. A landmark decision in this regard was the case of **Munjaz vs. Ashworth Hospital**. It is important that the seclusion practice is proportionate to the necessity for which it is used.

Vulnerable adult abuse

- ★ A vulnerable adult is defined as a person 'who is or may be in need of community care services by reason of mental or other disability, age or illness; and who is or may be unable to take care of him or herself, or unable to protect him or herself against significant harm or exploitation' (para. 2.4 No Secrets: Department of Health 2000).
- ★ The most frequent location of abuse is a person's home (41%) or a care home (34%). In 25% of cases family members are the perpetrators; care home staff abuse a further 25% of cases. Institutional abuse is often associated with poor training and inflexible treatment regimes.

- ★ The common types of abuse seen in vulnerable adults include was physical (30%), neglect (23%), financial abuse (20%), emotional/psychological abuse (16%) sexual abuse (6%). Neglect includes acts of omission such as withdrawing help causing patients to suffer.
- ★ The prerequisites for referring a vulnerable adult to safeguarding processes are shown in the attached figure. *Safeguarding Adults* (Department of Health 2011) describes six fundamental principles to safeguard vulnerable adults against abuse. These principles include
 1. Empowerment to allow person-led decisions and consent
 2. Protection, support and representation for those in greatest need
 3. Prevention of harm or abuse
 4. Proportionality and least intrusive appropriate response to the risk
 5. Partnerships to provide local solutions through community-based services
 6. Accountability and transparency when safeguarding an abused person



Child abuse

Risk factors associated with child maltreatment can be grouped in four domains:

i. Parent or caregiver factors:

- ★ Personality characteristics and psychological well-being (low self-esteem, an external locus of control (i.e., belief that events are determined by chance or outside forces beyond one's personal control), poor impulse control, depression, anxiety, and antisocial behavior)
- ★ History of maltreatment (about one-third of all individuals who were maltreated will subject their children to maltreatment)
- ★ Substance abuse

- ★ Attitudes and knowledge (Mothers who physically abuse their children have both more negative and higher than normal expectations of their children, as well as a poor knowledge regarding appropriate developmental norms)
- ★ Age (Teenage mothers and young parents abuse their children more often than mature parents.)

ii. Family factors

- ★ Specific life situations of some families—such as marital conflict, domestic violence, single parenthood, unemployment, financial stress, and social isolation—may increase the likelihood of maltreatment
- ★ The rate of child abuse in single parent households is 27.3 children per 1,000, which is nearly twice the rate of child abuse in two parent households.
- ★ In 30 to 60 percent of families where spouse abuse takes place, child maltreatment also occurs.

iii. Child factors

- ★ The rate of documented maltreatment was highest for children between birth and 3 years of age
- ★ Children with disabilities were 1.7 times more likely to be maltreated than children without disabilities

iv. Protective factors:

- ★ Emotionally satisfying relationships
- ★ A network of relatives or friends
- ★ Parents who were abused as children are less likely to abuse their own children if they have resolved internal conflicts and pain related to their history of abuse and if they have an intact, stable, supportive, and nonabusive relationship with their partner.

Finkelhor (1988) proposed a **traumagenic dynamics** model, which postulates that adverse effects of child sexual abuse depend on the presence or absence of four key factors. (1) Powerlessness (2) Betrayal (3) traumatic sexualization and (4) Stigmatization.

A **child abuse accommodation syndrome** has also been described. This refers to children/families susceptible for continuous on-going abuse without reporting them. The elements are

1. Secrecy related to the need to keep quiet due to the fear of the consequences
2. Helplessness with on-going threat of further abuse
3. Entrapment and accommodation with a destructive effect on personality development
4. Delayed and unconvincing disclosure at times of conflict with the family. This often results in rejection of the child's story and creates a damaging sense of being falsified if truth is revealed
5. Retraction of the disclosure due to a threat of disintegration of the family

DISCLAIMER: This material was developed from various revision notes assembled while preparing for MRCPsych exams. The content is periodically updated with excerpts from various published sources including peer-reviewed journals, websites, patient information leaflets and books. These sources are cited and acknowledged wherever possible; due to the structure of this material, acknowledgements have not been possible for every passage/fact that is common knowledge in psychiatry. We do not check the accuracy of drug-related information using external sources; no part of these notes should be used as prescribing information.

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